



Laboratory Investigation Report

Patient Name	: Soniya	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 23 Y 0 M 1 D /F	OP/IP No	: OP/99846
Max ID/Mobile	: 444907/8752005200	Collection Date/Time	: 31/Aug/2021 04:21PM
Lab ID	: 1266082100733	Receiving Date	: 31/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 31/Aug/2021

Clinical Biochemistry

Glycosylated Haemoglobin (HbA1C), EDTA Routine*

HPLC

Date	31/Aug/2021 04:21PM	Unit	Bio Ref Interval
Glycosylated Haemoglobin(Hb A1c)	5.7	%	4.27 - 6.07
Glycosylated Haemoglobin(Hb A1c) IFCC	38.78		
Average Glucose Value For the Last 3 Months	116.89		
Average Glucose Value For the Last 3 Months IFCC	6.47		

Interpretation The following HbA1c ranges recommended by the American Diabetes Association(ADA) may be used as an aid in the diagnosis of diabetes mellitus.

HbA1C(NGSP %)	HbA1C(IFCC mmol/mol)	Suggested Diagnosis
≥ 6.5	≥ 48	Diabetic
5.7 - 6.4	39 - 47	Pre- Diabetic
< 5.7	< 39	Non - Diabetic

HbA1C provides a useful index of average glycaemia over the preceding 6-8 weeks.

It is suggested that HbA1c is measured every 6 months in stable patients, every 3 months in patients with unstable metabolic control and every month in pregnancy.

Increased Glycated hemoglobin is a reflection of Hyperglycemia.



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CIN No.: L72200MH2001PLC322854

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Clinical Biochemistry

KFT Profile with Calcium,Uric Acid*, Serum

Date	31/Aug/2021 04:25PM	Unit	Bio Ref Interval
Urea Enzymatic Rate (Urease)	15.0	mg/dL	17.12 - 55.64
Creatinine Rate-Jaffe	0.41	mg/dl	0.61 - 1.24
eGFR MDRD	192.24	ml/min/1.73 m ²	
Uric Acid Enzymatic Trinder	4.18	mg/dl	2.6 - 8.0
Calcium (Total) ISE Indirect	8.9	mg/dl	8.9 - 10.3
Sodium ISE Indirect	138.2	mmol/L	136 - 144
Potassium ISE Indirect	4.32	mmol/L	3.6 - 5.1
Chloride ISE Indirect	103.2	mmol/l	101-111
Bicarbonate ISE Indirect	20.8	mmol/l	22-32

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs ≤ 60 ml / min /1.73 m².MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min /1.73 m² - Kidney failure



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Clinical Biochemistry

Liver Function Test Profile,Serum*

Date	31/Aug/2021 04:25PM	Unit	Bio Ref Interval
Total Protein Biuret	6.80	g/dl	6.5 - 8.1
Albumin BCP	4.8	g/dl	3.5 - 5.0
Globulin Calculated	2.0	g/dl	2.3 - 3.5
A.G. ratio Calculated	2.5		1.2 - 1.5
Bilirubin (Total) Diazo	0.28	mg/dl	0.3 - 1.2
Bilirubin (Direct) Diazo	0.11	mg/dl	0.1 - 0.5
Bilirubin (Indirect) Calculated	0.17	mg/dl	0.1 - 1.0
SGOT- Aspartate Transaminase (AST) UV without P5P	20.12	U/L	10 -35
SGPT- Alanine Transaminase (ALT) Kinetic Rate using LDH	15.14	U/L	< 40
Alkaline Phosphatase PNP AMP Buffer	96.76	U/L	32 - 91
GGTP (Gamma GT), Serum Enzymatic Rate	15.0	U/L	7 - 50

Kindly correlate with clinical findings

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Clinical Biochemistry

Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	31/Aug/2021 04:23PM	Unit	Bio Ref Interval
APTT	33.3	Sec	24.9 - 33.3
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the “intrinsic coagulation” pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - ‘APTT mixing study’, ‘Lupus Anticoagulant test’, ‘specific Factor(s) assay’ may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	31/Aug/2021 04:25PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	12.8	Sec	12.1 - 15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.13		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the “extrinsic coagulation” pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - ‘PT mixing study’, ‘specific factor(s) assay’ may be added on for further evaluation.



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Hematology

CBC (Complete Blood Count)*, Whole Blood EDTA

Date	31/Aug/2021 04:25PM	Unit	Bio Ref Interval
Haemoglobin	12.20	g/dl	12.0 - 15.0
Modified cyanmethemoglobin			
Packed Cell, Volume	41.8	%	40-50
Calculated			
Total Leucocyte Count (TLC)	10.17	10~9/L	4.0-10.0
Electrical Impedance			
RBC Count	5.78	10~12/L	3.8-4.8
Electrical Impedance			
MCV	72.3	fL	83-101
Electrical Impedance			
MCH	21.1	pg	27-32
Calculated			
MCHC	29.2	g/dl	31.5-34.5
Calculated			
Platelet Count	288.8	10~9/L	150-410
Electrical Impedance			
MPV	12.49	fL	7.8-11.2
Calculated			
RDW	15.5	%	11.5-14.5
Calculated			

Differential Cell Count

VCS / Light Microscopy

Neutrophils	66	%	40-80
Lymphocytes	24	%	20-40
Monocytes	06	%	2-10
Eosinophils	04	%	1-6
Basophils	00	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	6.71	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.4	10~9/L	1.0-3.0
Absolute Monocyte Count	0.61	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.41	10~9/L	0.02-0.5



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Hematology

Kindly correlate with clinical findings

*** End Of Report ***


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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test	Negative
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Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Rapid Card Test - HIV I & II*, Serum

Immunochromatography

Rapid Card HIV I & II	Negative
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Comment This is only a screening test. All samples detected reactive must be confirmed by using HIV RNA PCR / three different methods.
A non- reactive test result does not exclude the possibility of exposure to an infection with HIV (Window Period) or low viral load.

Advice:

1. HIV by 3 different methods
2. Confirmatory HIV RNA Quantitative PCR.



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Serology

Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis)
Slide Flocculation

Non Reactive

Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.



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Serology**Viral Markers (Rapid)**

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - Hepatitis B Surface Antigen*, Serum

Immunochromatography

Rapid Card HBsAg Negative

Comment**Interpretation**

This is only a screening test.

All reactive samples should be confirmed by 'HBsAg confirmatory test or HBV DNA PCR'.

A non – reactive does not exclude the possibility of exposure to infection with HBV (window period)

False positive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

Advise: Confirmatory test 'HBsAg Confirmatory Quantitative' test followed by 'HBV DNA quantitative PCR'

Kindly correlate with clinical findings

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